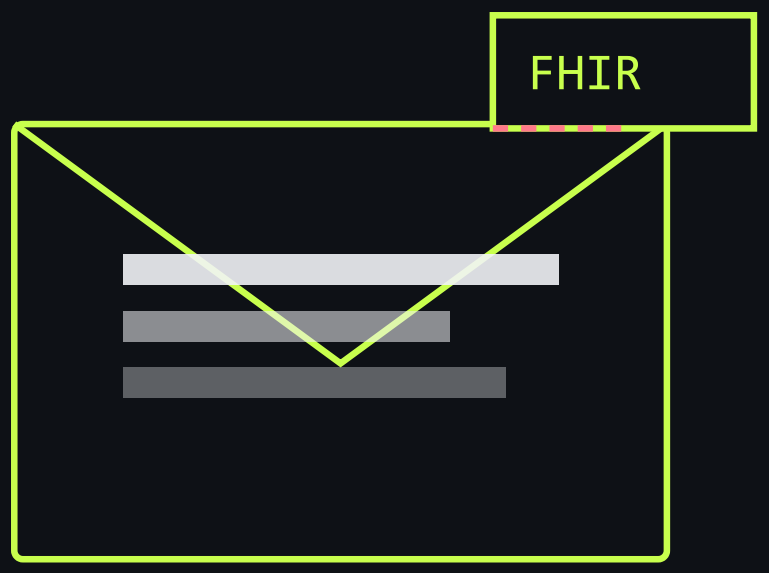
The record arrives as a letter.

Five slides on what breaks in UK care data. Five on the build that closes it.

FHIR R4 | SNOMED CT | NHS NUMBER



NOT DATA. YET.

Four records. Four owners.

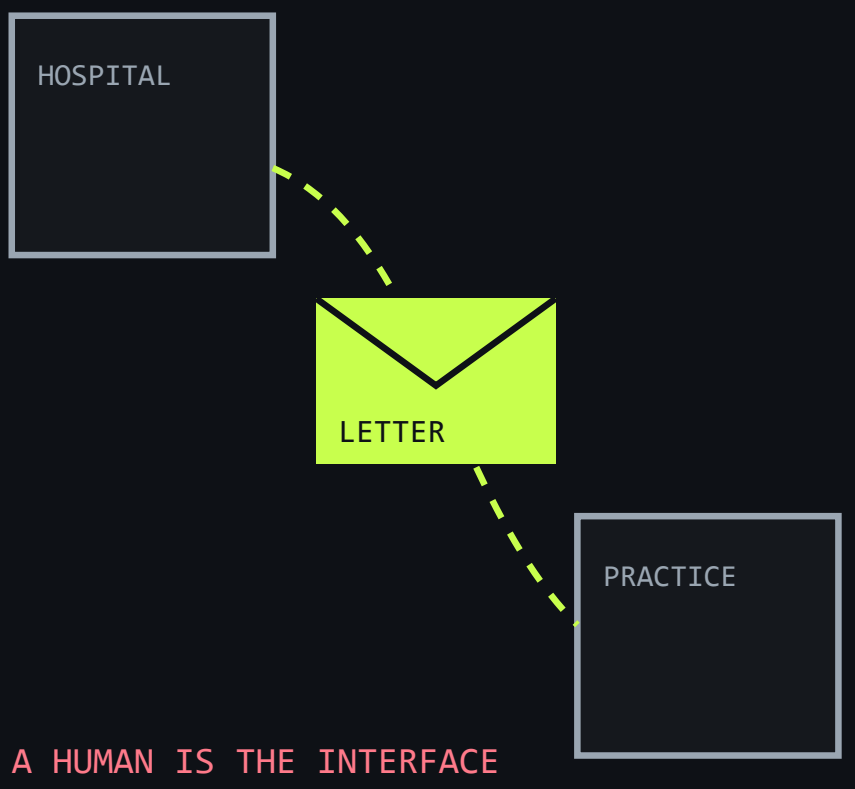
- **GP practice** — EMIS Web or SystmOne holds the lifelong record
- **Hospital trust** — its own EPR, its own login
- **Pharmacy** — dispensing records, usually unlinked
- **National** — NHS number, Spine, GP Connect: plumbing, not the record



So what joins them?

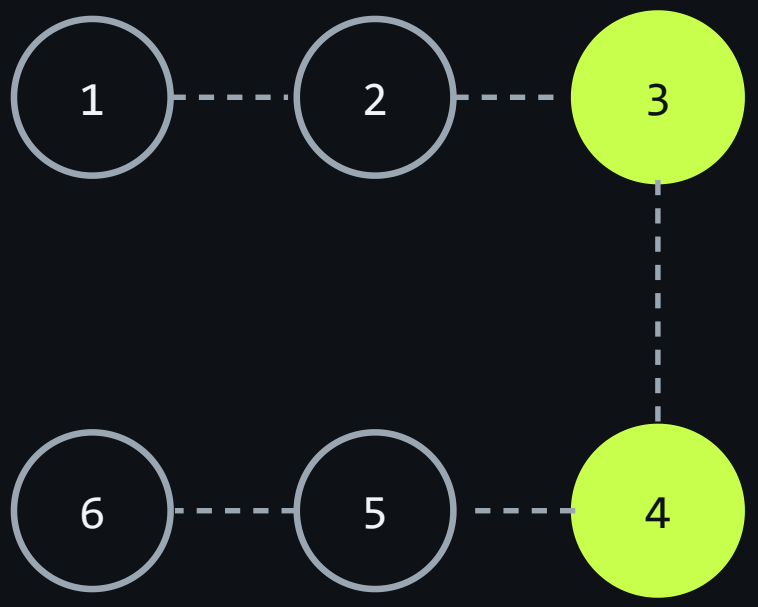
Discharge summaries and clinic letters, arriving as documents.

A PDF is not a database row. Someone must read it, decide what it means, and code it — before the record knows anything at all.



Six hands.
Two are manual.

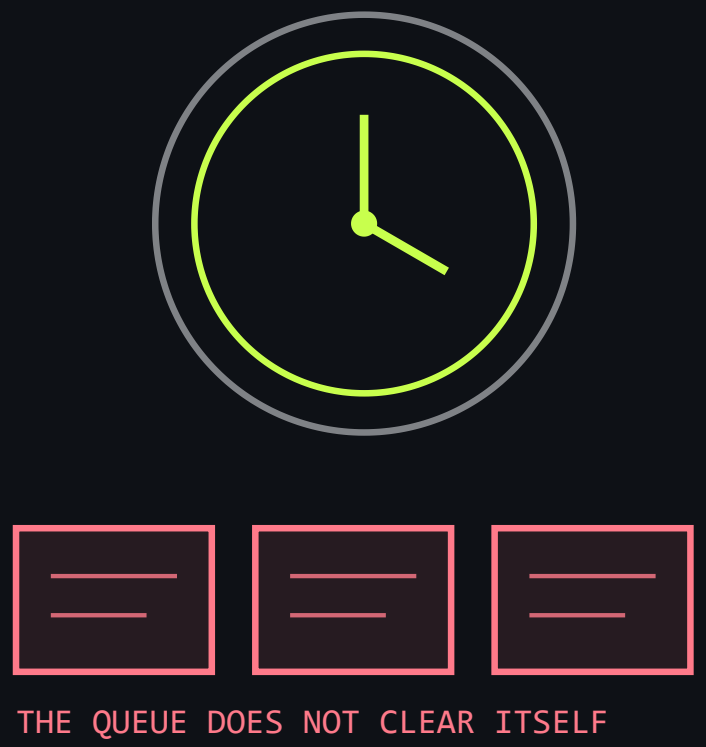
- Arrives →
- filed as a document →
- **read and coded by a person →**
- **filed to the record, by a person →**
- recall depends on the code →
- clinician acts, days later



STEPS 3 AND 4 ARE DONE BY HAND

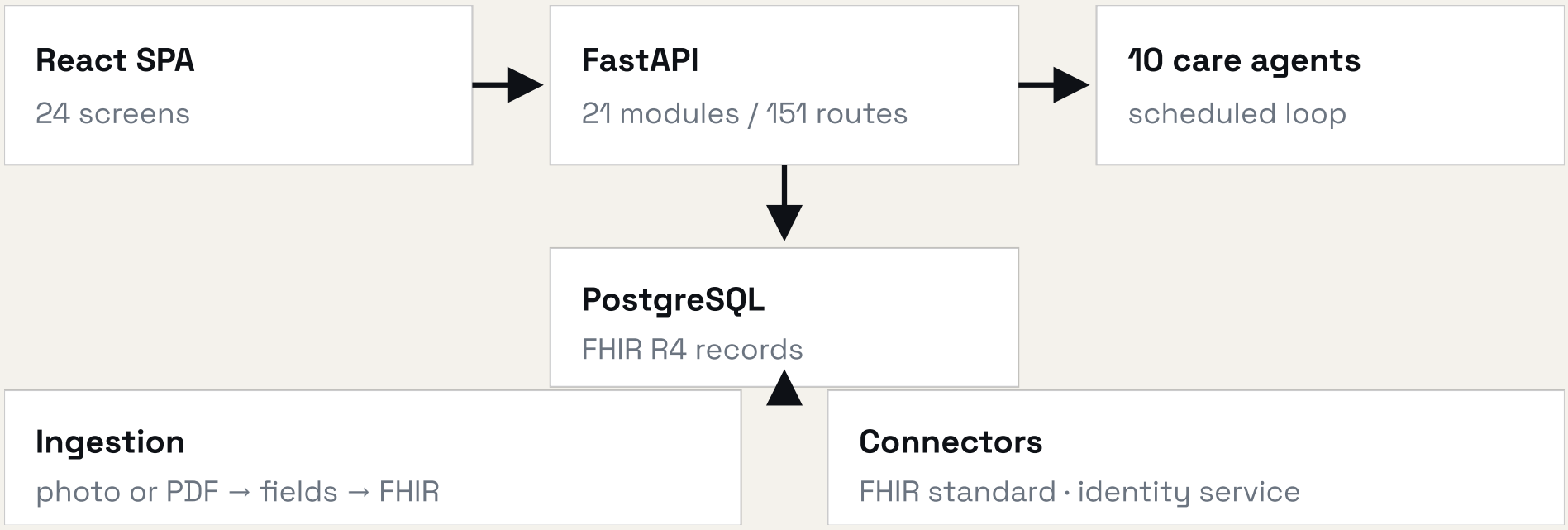
What that costs

- **Coding lag** — on paper, not in the searchable record
- **Recall misses** — queries run on codes
- **Duplication** — typed again elsewhere
- **Delay** — an inbox result is an absent result



THE QUEUE DOES NOT CLEAR ITSELF

Structure of the running code

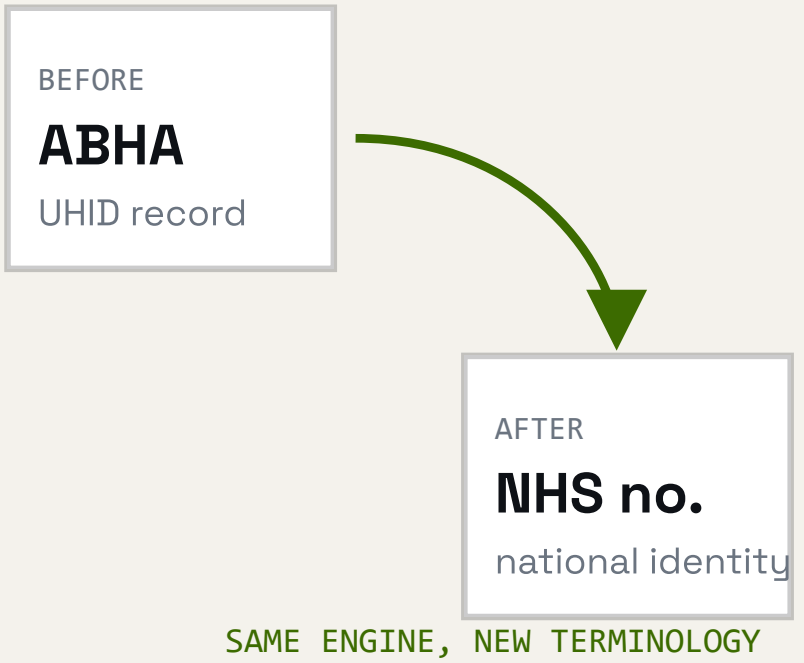


One source of truth — the agents read the same records clinicians write.

Tenant-scoped — the JWT carries the practice; every query is filtered.

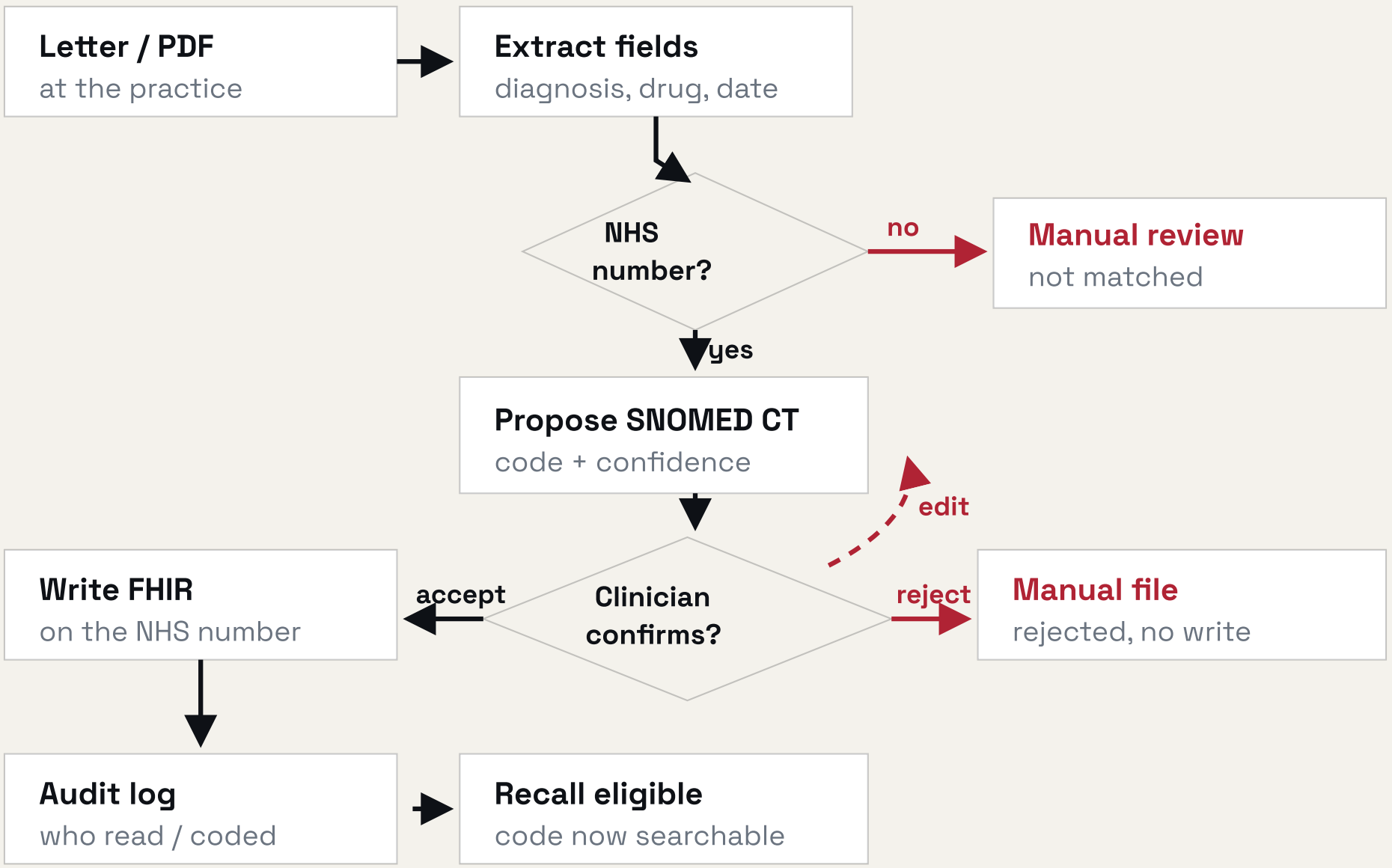
Written for India. Mapped for the UK.

IN THE CODE	UK
ABHA / UHID	NHS number
ICD-10	SNOMED CT
FHIR R4	UK Core
DPDP 2025	UK GDPR



SAME ENGINE, NEW TERMINOLOGY

One letter, three outcomes



WHAT EXISTS TODAY

151
REST endpoints
across 21 modules

24
clinical screens
React + TypeScript

10
care agents
on a schedule

33/45
build tasks
complete

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patient intake · appointments · medication adherence · follow-up · labs · pharmacy · insurance · risk prediction · family care · voice care

What it will not do — or claim

- **No diagnosis** — no autonomous clinical decision
- **No shared pool** — no cross-practice data
- **No silent writes** — every fact attributable
- **No certificates** — DTAC, DSPT, DCB0129 not held



SEE THE LOOP — SIX SCREENS, SAMPLE DATA

19p1362.github.io/portfolio/healthbridge

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